Colonialism, Public Health, and Race:

Epidemic Cholera and the Consolidation of American Power in the Philippines, 1898-1904

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Introduction

For American officials in the newly conquered Philippines, the outbreak of epidemic cholera in 1902 arrived at a crucial moment of transition. While the Philippine-American War was not officially concluded until July 1902, the Filipino insurgent movement was nevertheless withering. As a result, the archipelago was undergoing a transition from military rule to civil rule. During this same period, another transition was occurring in the minds of American officials: Filipinos were being racially transformed from enemy combatants to American subjects. Just as significantly, American public health officials were abandoning older miasmatic theories of disease in favor of the germ theory of disease. While the United States initially relied on military force and territorial occupation to control the Philippines, the public health policies enacted by American officials during the 1902-1904 cholera epidemic transitioned to newer methods of colonial control, which often intersected with advances in medical thought. American actions against cholera did reduce the deadliness of the disease, but ultimately the most important effects of these actions were the reinforcement of colonial functionaries' surveillance powers and the creation of a racialized understanding of Filipinos as innately uncivilized, unclean, and resigned to disease and death.

Cholera is an intestinal disease caused by the bacterium *Vibrio cholerae*, which is typically transmitted through contaminated water. Within five days of ingesting the bacterium, those who are infected develop severe diarrhea and vomiting, leading to potentially fatal dehydration and electrolyte imbalances. By the beginning of the Philippine-American War in 1898, American doctors had identified the *Vibrio cholerae* bacterium and the symptoms it caused. English medical scientist John Snow had been the first to link cholera with contaminated water after investigating an outbreak in London in 1849. Despite Snow's microbial theory and

the isolation of *Vibrio cholerae* by Italian scientist Filippo Pacini in 1854, miasmatic theories remained in vogue until 1883, when German scientist Robert Koch isolated *Vibrio cholerae* and forcefully argued in favor of the germ theory of disease.[1] While American doctors in the Philippines could recognize cholera and its symptoms, they were almost completely ignorant of effective treatments for the disease. While modern therapies include antibiotics, oral rehydration, and IV fluids, treatments during the 1902-1904 epidemic revolved around the use of strong acids to flush the bacterium out of the patient's system. These treatments were not only ineffective but also dangerous, sometimes destroying the throat and stomach lining of the patient.[2] While American medical officials understood cholera's symptoms and the importance of clean water in preventing the disease's spread, they were almost entirely powerless to treat those who were already infected.

The Philippines experienced cholera epidemics throughout the 19th century, but the 1902-1904 outbreak was particularly deadly and spread at a rapid pace due to the dislocation caused by the Philippine-American War. The epidemic officially began on March 20, 1902, when four Filipinos were diagnosed with the disease at Manila's San Juan de Dios Hospital. American officials quickly blamed the appearance of cholera on a shipment of contaminated vegetables that had recently arrived from southern China, been thrown into Manila Bay, and been eaten by impoverished Filipinos.[3] While this hypothesis is possible, it should not be taken at face value; it aligns very neatly with American stereotypes of Filipinos as unclean in their eating habits. It is just as likely that cholera was introduced by one of the thousands of sailors and laborers who

1. Christopher Hamlin, *Cholera: The Biography* (Oxford: Oxford University Press, 2009), 9.
2. Willie Ong, "Public Health and the Clash of Cultures: the Philippine Cholera Epidemics," in *Public Health in Asia and the Pacific: Historical and Comparative Perspectives*, ed. Milton J. Lewis and Kerrie L. MacPherson (London: Routledge, 2008), 211.
3. Dean C. Worcester, *A History of Asiatic Cholera in the Philippine Islands* (Manila: Bureau of Printing, 1908), 15.

worked at Manila's bustling port. Regardless of its exact origins, the disease rapidly spread through the city. At the end of March, 84 cases and 65 deaths were reported, [4] and over the next month the city would record 496 new cases, 400 of them resulting in death.[5]

Despite American efforts to enforce a strict quarantine in Manila, the epidemic reached all but the tiniest islands in the archipelago by July 1902.[6] The disease raged through the summer and peaked during the first week of October, when approximately 9,000 cholera cases were recorded by American officials.[7] The first and most severe wave of the epidemic concluded in February of 1903, but another surge occurred between May 1903 and February 1904.[8] Perhaps the most staggering aspect of the 1902-1904 epidemic was the 65% death rate among the infected.[9] This death rate was exacerbated by several factors that will be examined later in this paper, including: a complete lack of medical treatment for cholera, counterproductive measures enforced by American officials, and the aftereffects of four years of warfare in the Philippines. All of these factors contributed to the official (and probably incomplete) death toll of 166,252 by the end of the epidemic in early 1904.[10] The epidemic was indisputably a tragedy, and scholars have suggested that it was responsible for almost a third of excess Filipino mortality in the year 1902 alone.[11]

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4. *Public Health Reports* 17, no. 19 (May 1902): 1091.
5. *Public Health Reports* 17, no. 25 (June 1902): 1472.
6. Andrew Cliff and Matthew Smallman-Raynor, "The Philippines Insurrection and the 1902-4 Cholera Epidemic: Part I-Epidemiological Diffusion Processes in War," *Journal of Historical Geography* 24, no. 1 (January 1998): 77.

7. Cliff and Smallman-Raynor, "Diffusion Processes in War," 74.
8. Cliff and Smallman-Raynor, "Diffusion Processes in War," 74.
9. The case fatality rate given here was calculated using official case and death numbers found in Worcester, *Asiatic Cholera*, 16, 21.
10. Worcester, *Asiatic Cholera*, 21.

11. Ken De Bevoise, *Agents of Apocalypse: Epidemic Disease in the Colonial Philippines* (Princeton, NJ: Princeton University Press, 1995), 184.

American Characterizations of the "Filipino Race"

Before examining how American conceptions of race influenced colonial cholera policy in the Philippines, it is first necessary to summarize what racial characteristics American officials attributed to Filipinos. Even within the short period lasting from 1898 to 1904, American attitudes toward Filipinos varied considerably. Paul Kramer convincingly argues that during the Philippine War, Americans regarded Filipinos as complete savages, pointing out how United States soldiers contrasted the conventional and supposedly civilized warfare favored by Americans with Filipino guerilla tactics that were derided as uncivilized and indicative of Filipino racial inferiority.[12] However, by the war's end, many officials abandoned this wartime rhetoric and acknowledged that Filipinos were not a wholly savage race; while it might take decades of American tutelage, Filipinos had the potential to eventually govern themselves. This position is summarized in the Philippine Commission's 1900 report to President McKinley, in which the authors advised: "the power of self-government can not be assumed [by Filipinos] without considerable previous training and experience under the guidance and tutelage of an enlightened and liberal sovereign power."[13] Even after the war, racial attitudes varied among American officials. In his capacity as Civil Governor of the Philippines, William Howard Taft was optimistic about the potential of the Filipino race, as he stated in 1903. Just as Americans were exceptional colonizers, Taft characterized Filipinos as exceptional subjects: "contrast the Filipinos with other Malays and the Oriental peoples, and I ask you to name a people offering more opportunities for development along the lines which American ideals require than the

12. Paul A. Kramer, *The Blood of Government: Race, Empire, the United States, and the Philippines* (Chapel Hill: University of North Carolina Press, 2006), 138.

13. Bureau of Insular Affairs, *Report of the Philippine Commission to the President*, vol. 1 (Washington D.C.: Government Printing Office, 1900), 182.

people of these islands."[14] Dean Worcester, Secretary of the Interior from 1901 to 1913 and director American actions against cholera, saw things differently. Worcester stridently argued against independence for the Philippines, no matter how far off; for him, Filipinos were not a civilization, but "a variegated assemblage of different tribes and peoples," which would descend into anarchy without strong American rule.[15] Despite these differing attitudes, American officials universally agreed that Filipinos were not as civilized as Americans and would require years of "tutelage" before any consideration of independence.

While American officials argued for Filipino racial inferiority on many grounds, perceptions of hygiene played a key role in the process of racialization. During the early American colonial period in the Philippines, Americans and Filipinos were differentiated not just by their skin tone and other supposed physical differences but also by their hygienic habits. Americans placed a special emphasis on Filipinos' defecation habits and sewage systems (or lack thereof). Around 1900, most Filipinos had no access to outhouses, let alone the complex sewer systems of American cities like New York. When researchers from the Bureau of Health conducted a survey of Taytay, a highly developed town by Filipino standards in 1909, they reported that only a quarter of households had outhouses; those without them used either outdoor pits or vessels within the house.[16] Another American sanitary worker linked the "lack of intelligence displayed by the average Filipino" to the town of Surigao being "invariably covered with filth of all kinds, human excrement included."[17] Disgusted accounts of Filipino towns by

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14. Bureau of the Census, *Census of the Philippine Islands: Taken Under the Direction of the Philippine Commission in the Year 1903*, vol. 1 (Washington D.C.: Government Printing Office, 1905), 529.

15. *Report of the Philippine Commission*, vol. 1, 182.

16. Paul Clements, "Medical Survey of the Town of Taytay: IX General Sanitary Conditions," *Philippine Journal of Science* 4B, no. 4 (1909): 251-52.

17. Henry du Rest Phelan, "Sanitary Service in Surigao, a Filipino Town in the Island of Mindanao," *Journal of the Association of Military Surgeons* 14 (1904): 5-6.

American officials, teachers, and soldiers abound, but what is more important than the details of these accounts is the way they link Filipino hygiene to a lack of maturity and control. Warwick Anderson, an Australian historian of science and medicine, has argued that this supposed lack of excremental control led Americans to label Filipinos as "promiscuous defecators" who were both unsightly and dangerous.[18] Open defecation not only offended American eyes and noses but also enabled the spread of disease. By contrasting Filipinos' supposed lack of hygienic control with the standards favored by Americans, colonial sanitary workers created the perception that Filipinos were racially unclean and unable to control their bodies.

Beyond the accounts of sanitary inspectors, American perceptions of Filipinos as dangerously unclean were furthered by the popularization of the germ theory of disease. While the accounts of sanitary inspectors provided disgusting images of individual Filipinos, it was the theoretical work of American medical workers that led Americans to view Filipinos as *universally* and *racially* unclean. This transition from the particular to the universal was enabled by American sanitary workers' embrace of the germ theory of disease. Before the germ theory of disease became generally accepted, illnesses were blamed on environmental miasmas rather than specific microorganisms. During the period of Spanish rule in the Philippines, for example, Spanish doctors blamed sickness among Europeans on the tropical climate rather than individual pathogens.[19] However, the adoption of germ theory would shift the blame for disease from Filipino geography to Filipino people themselves. Instead of fearing the midday heat, Americans began to associate tropical diseases with Filipinos, who were regarded as reservoirs of infection. Military surgeon Weston Chamberlain warned that even those Filipinos who appeared healthy

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18. Warwick Anderson, *Colonial Pathologies: American Tropical Medicine, Race, and Hygiene in the Philippines* (Durham, NC: Duke University Press, 2004), 128.
19. Anderson, *Colonial Pathologies*, 76-77.

were potentially dangerous; his research concluded that more than ninety percent of Filipinos had intestinal parasites, but most showed no symptoms of infection.[20] Likewise, Major Charles Woodruff advised Americans "to consider all apparently healthy native soldiers as possible sources of fatal infection to whites," after Filipino scouts were found to asymptomatically carry malaria parasites.[21] Aided by the insights of germ theory, Americans characterized the entire Filipino race as dangerously unclean; even if Filipinos overcame their supposedly unclean cultural habits, they still presented an unseen and microbial threat to Americans.

The final piece necessary to understand how race functioned in the 1902-1904 cholera epidemic is to view how Americans, particularly American medical officials, regarded themselves as benevolent figures bringing Anglo-Saxon advancement to an inferior race. From the very beginning of the United States' invasion of the Philippines, American intervention was seen in racial terms. Rudyard Kipling's famous 1899 poem "The White Man's Burden" was addressed to Americans, encouraging them to take up the civilizing mission already embraced by the British Empire. When the United States invaded the Philippines, the conflict against Spain was viewed in racial terms, with Americans cast as liberators. Addressing Congress in 1900, Senator Albert Beveridge blamed Filipino inferiority on both native ignorance and Spanish misrule:

> [Filipinos] know nothing of practical government except as they have witnessed the weak, corrupt, cruel, and capricious rule of Spain….What alchemy will change the oriental quality of their blood and set the self-governing currents of the American pouring through their Malay veins? How shall they, in the twinkling of an eye, be exalted to the heights of self-governing peoples which required a thousand years for us to reach, Anglo-Saxon though we are?[22]

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20. Weston P. Chamberlain, E. D. Kilbourne, and H. D. Bloombergh, "Examination of Stools and Blood Among the Igorots at Baguio, Philippine Islands," *Philippine Journal of Science* 5B, no. 5 (1910).

21. Anderson, *Colonial Pathologies*, 91.

22. Albert J. Beveridge, "The Philippines," in *Orations of American Orators*, vol. 3, ed. Richard Garnett (New York: Fifth Avenue Press, 1900), 388.

In place of Spanish despotism, Catholicism, and racial mixture, Americans saw it as their duty to deliver democracy, Protestantism, and Anglo-Saxon racial purity. American medical workers also saw their duty in racial terms, often portraying themselves as saviors of the backward Filipino race. Victor Heiser, Philippine Director of Health, wrote that he strove "to transform the Filipinos from the weak and feeble race they were into the strong, healthy, and enduring people they might become."[23] Heiser's attitude nicely sums up American racial thought concerning Filipinos: they were an inferior race, but capable of improvement if they accepted an indefinite period of Anglo-Saxon tutelage.

If the accounts of American officials on Filipino racial characteristics and inferiority seem inconsistent to the reader, this is because they were. Officials in the United States, such as Senator Beveridge, often derided Filipinos as inferior by using abstract racial types and argued that Filipinos were inferior to Anglo-Saxons because of their racial "blood." Other officials, particularly those who actually worked in the Philippines, used supposed Filipino technological backwardness and sanitary ignorance as the basis for Filipino racial inferiority. The racialization of Filipinos by American officials was ultimately inconsistent because it was more of an excuse for American conquest than an internally coherent system of racial classification. For American officials, the only consistent characteristic of the Filipino was his inferiority; this is why colonial officials in the Philippines so often blended race, culture, civilization, and medical knowledge into a single concept that they often called race.

Deploying Race in the 1902-1904 Cholera Epidemic

23. Victor Heiser, *An American Doctor's Odyssey: Adventures in Forty-five Countries* (New York: Norton, 1936), 38.

Throughout the 1902-1904 cholera epidemic, American colonial officials relied on their racial understanding of Filipinos as unclean, immature, and fatalistic when formulating policies to stop the spread of disease. While some of these policies curbed the spread of cholera and others were unsuccessful, all of them hinged on the assumption that Filipinos were dangerous carriers of disease. Furthermore, colonial anti-cholera policies placed the blame for disease solely on Filipinos, neglecting the role that American officials had in the epidemic.

The destruction of entire neighborhoods of Filipino *nipa* houses provides one of the most striking examples of how American anti-cholera policies were enforced along racial lines. When cholera broke out in Manila in 1902, the *nipa* hut was by far the most common form of housing in the Philippines; the 1903 Philippine Census found that 94 percent of Filipino dwellings "were of nipa or similar construction."[24] These homes were typically constructed with a bamboo frame, stilts that raised the house off the ground, and walls and roofing made from the leaves of the *nipa* palm. While the *nipa* homes served as reliable and affordable dwellings for most Filipinos, Americans derided them as liable to "[become] infested with vermin and insect life."[25] Additionally, *nipa* homes were often abandoned or rebuilt after five years. American officials saw impermanent dwellings as another sign of Filipino inferiority since they associated advanced civilization with lasting buildings, preferably made of stone. Given American officials' association of *nipa* homes with unsanitary living conditions, it is unsurprising that they were targeted almost immediately after cholera was discovered in Manila in 1902. When some of the initial cholera cases were traced to the Farola district in Manila, Secretary of the Interior Dean Worcester immediately recommended "that this district, which was covered for the most part

24. US Census Bureau, *1903 Philippine Census*, vol. 3, 703.
25. US Census Bureau, *1903 Philippine Census*, vol. 3, 703.

with filthy and almost worthless shacks, be burned over and its inhabitants be transferred to the detention camp."[26] On March 28, after resisting Worcester's orders for three days, Manila's Commissioner of Public Health oversaw the burning of the 125 *nipa* houses that made up the Farola district, described by one newspaper as, "the filthiest blot in all Manila."[27] While this measure did not prevent the spread of cholera, it did demonstrate how Worcester's perception of Filipino dwellings as unclean outweighed his understanding of disease. By the time Farola was burned in late March, there were already more than one hundred cholera cases, many of them detected in other districts in Manila.[28] Still, Worcester initially regarded the other districts of the city, particularly the colonial center, with its imposing stone buildings, as beyond suspicion, even if the area was renowned for its commerce between American soldiers and Filipino prostitutes. Through his orders to burn the Farola district, Worcester demonstrated how American officials instantly associated Filipinos and their customs with disease while simultaneously exonerating Americans because of their supposedly civilized habits.

The destruction of *nipa* huts as suspected sites of disease was not limited to Manila. Government Order 66, issued on March 25, commanded that the *nipa* homes of any person infected with cholera be destroyed.[29] However, the order allowed homes made of permanent materials to be spared as long as the inhabitants allowed American sanitary workers to disinfect them.[30] By sparing houses made of permanent materials, which often belonged to colonial officials or Filipino elites, American officials again demonstrated how they viewed most

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26. Worcester, *Asiatic Cholera*, 24.

27. "The Burning of Farola," *Manila Times*, March 30, 1903.
28. Worcester, *Asiatic Cholera*, 16.
29. "Chaffee's Cholera Order," *Manila Times*, March 27, 1902.
30. Reynaldo C. Ileto, "Cholera and the Origins of the American Sanitary Order in the Philippines," in *Imperial Medicine and Indigenous Societies*, ed. David Arnold (Manchester: University of Manchester Press, 1988), 136.

Filipinos as irredeemably disease-ridden. While the houses of colonial officials and Filipino elites who cooperated with Americans were seen as redeemable, the impermanent and supposedly uncivilized nature of *nipa* houses made them innately disposable and disease-ridden.

Another American anti-cholera policy with striking racial aspects was the creation of cholera hospitals and detention camps. Within a month of the first cholera cases in Manila, Worcester ordered the creation of two hospitals and accompanying detention camps in the city, one at San Lazaro and another at Santa Mesa.[31] These facilities were necessary because any person who contracted cholera and was discovered by sanitary officials was immediately sent to a cholera hospital. Anyone who had been in contact with the infected person was taken to one of the city's detention camps, and their home was either burned or disinfected. This policy of forced removal was not unfamiliar to American officials and had striking parallels to tactics used during the Philippine War. In December of 1901, for example, the residents of Laguna and Batangas provinces were "reconcentrated" into towns and forced to abandon their crops under suspicion that they were aiding Filipino rebels.[32] While the detention camps established in Manila were not policed as harshly as those in war zones, even Worcester admitted that they were inadequate and barely livable spaces.[33]

The forced removal of those infected with cholera was racially motivated in both practice and theory. On a practical level, detention in hospitals and camps was only a reality for Filipinos. Even though nearly one hundred American soldiers had contracted cholera by mid-April, largely due to unauthorized mixing with Filipinos in bars and brothels, these soldiers were

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31. Worcester, *Asiatic Cholera*, 19-20.
32. De Bevoise, *Agents of Apocalypse*, 179.
33. Worcester, *Asiatic Cholera*, 25.

not placed in detention camps but instead given medical care by the United States Army.[34] Likewise, no evidence exists to suggest that any American officials or their family members ended up in detention camps. In fact, as Ileto writes, "Americans, Spaniards, and Chinese had well-equipped hospitals catering specifically for them."[35] It was only Filipinos in Manila who faced the prospect of forced detention while the camps were used between March and mid-May. American officials like cited Filipino reluctance to enter the camps as evidence of "hysteria" while simultaneously admitting that those who were forced into the camps died "four times out of five."[36] The decision by American health officials to only detain Filipinos provided a clear example of how anti-cholera policies were enforced on a racial basis and led to disproportionate infection, suffering, and death for Filipinos.

On a more theoretical level, forcible detentions in the San Lazaro and Santa Mesa camps demonstrated how American officials viewed Filipinos as immature and fatalistic. The policy of forced detention emerged from the American view of Filipinos as a weak race resigned to death and disease. Philippine Director of Health Victor Heiser, for example, claimed that Filipinos "were apparently contented in their ignorance and poverty, and resigned to their many ailments"[37] In contrast, Americans were viewed as strong examples of the Anglo-Saxon race who struggled against cholera rather than abandoning hope. This dichotomy of fatalistic Filipinos and striving Anglo-Saxons is visible in popular accounts of the epidemic, such as an article in the *Manila Times* that claimed Filipinos sometimes refused treatment once placed in hospitals, while Americans accepted treatment and "had none of that helplessness or despair so

34. De Bevoise, *Agents of Apocalypse*, 178.
35. Ileto, "American Sanitary Order," 128.
36. Heiser, *Doctor's Odyssey*, 106.
37. Heiser, *Doctor's Odyssey*, 39.

characteristic of the native."[38] American officials saw their decision to force infected Filipinos to receive experimental treatments and isolate in the detention camps as a humanitarian and benevolent. In their eyes, they were delivering Anglo-Saxon medical expertise and hope to a Filipino race that they characterized as primitive and completely accepting of death.

By focusing almost exclusively on Filipinos as the vectors of disease during the 1902-1904 cholera epidemic, American officials became blinded to the role of American soldiers in spreading cholera, especially early in the epidemic. After the initial discovery of cholera in March 1902, American soldiers were certainly responsible for some of the disease's spread, particularly in the Manila area. By mid-April, ninety cases of cholera were recorded among American soldiers, and medical officers blamed these infections on the violation of quarantine measures.[39] Soon after, military regulations were zealously enforced, more due to fears over troop strength than out of concern for rising fatalities among Filipinos. While American soldiers were not entirely responsible for the spread of cholera on Luzon, they surely played a role; soldiers accounted for roughly one-sixth of total cases by the end of April.[40] However, the most significant effect of the American military on the epidemic was how it facilitated the spread of cholera among the disparate islands of the Philippine Archipelago. In mid-April, a military transport ship brought cholera from Manila to Nueva Caceres, an important port in southern Luzon.[41] From Nueva Caceres, the epidemic quickly spread to the southern islands of the Philippines through American and Filipino boat traffic. Again, the blame for the spread of cholera cannot be placed entirely on the United States military, but its role was significant. While Filipinos were the source of inter-island traffic just as often as Americans, the scale and

38. "Cholera Mocks Efforts of Science," *Manila Times*, September 25, 1902.
39. De Bevoise, *Agents of Apocalypse*, 178.
40. Worcester, *Asiatic Cholera*, 16.
41. Cliff and Smallman-Raynor, "Diffusion Processes in War," 78.

speed of American military ships made them more dangerous in spreading cholera. Instead of transporting small loads of cargo and a few people, American ships carried hundreds of troops to the farthest islands of the Philippines, all at a rapid pace. Although the American military was not the sole vector in the spread of cholera in the 1902-1904 epidemic, American officials almost completely neglected the role that troop transport played in spreading the disease beyond Manila.

Beyond the immediate role of American troops in spreading cholera, American officials also failed to consider how the conditions of the Philippine-American War had made Filipinos particularly susceptible to cholera. The strikingly high fatality rate (roughly 65%)[42] during the 1902-1904 epidemic was partially due to the shortage of food that Filipinos suffered throughout the islands. In all phases of the Philippine War, control of the food supply played a crucial role in the American attempt to capture the archipelago. Soon after the war broke out in 1899, Filipinos were forbidden to move beyond their towns of residence without passes, severely limiting the flow of rice and other dietary staples.[43] Inter-island transport was also severely limited, and Filipinos struggled with hunger as it became difficult to transport rice from producing areas to those with shortages. As the war took a turn toward guerilla fighting in 1900, the situation grew even worse; storehouses in areas with guerilla activity were indiscriminately destroyed, and agricultural production came a halt.[44] While American offensives against the guerillas had been reduced by the time cholera was discovered in March 1902, food production and distribution remained in chaos. Another American tactic that made Filipinos particularly susceptible to cholera was the policy that soldiers called "reconcentration." Reconcentration was used primarily from 1901 to 1902 in an effort to finally root out Filipino resistance by controlling

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42. Worcester, *Asiatic Cholera*, 16, 21.
43. Worcester, *Asiatic Cholera*, 21.
44. De Bevoise, *Agents of Apocalypse*, 63.

the movement of people and goods within guerilla-controlled areas. Attempting to isolate Filipino insurgents, American troops in provinces like Batangas forced Filipino civilians to abandon their crops, homes, and possessions and move into garrisoned towns that functioned as concentration camps.[45] Reconcentration created an atmosphere perfect for the spread of disease: conditions were cramped, sanitation was almost impossible, and malnutrition was worsened as American soldiers destroyed any crops that might have supported Filipino insurgents. Addressing Congress, even President McKinley admitted that the strategy of reconcentration, at that time being by the Spanish in Cuba, "was not civilized warfare; it was extermination."[46] While American officials blamed Filipino uncleanliness for the spread of cholera, they neglected to consider how American policies such as reconcentration and crop destruction created the perfect conditions for an epidemic and increased susceptibility to disease.

Since American officials regarded Filipinos as racially inferior, they refused to take Filipino customs into account when formulating and enforcing anti-cholera policies such as forced detainment, experimental medical treatment, and mandatory cremation. This near-complete disregard for Filipino culture ultimately worsened the 1902-1904 epidemic by producing fear, concealment, and resentment among Filipinos. One of the most egregious oversights American officials made was the forced detainment and isolation of the infected. Beyond the obvious fear created by taking the infected from their homes, the strict isolation that the infected had to endure in Manila's detainment camps directly conflicted with Filipino habits. For Filipinos, it was customary to visit any family member or friend who fell ill in order to

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45. Glenn May, "Concentration and Conquest," in *Battle for Batangas: A Philippine Province at War* (New Haven, CT: Yale University Press, 1991).

46. *Papers Relating to the Foreign Relations of the United States, with the Annual Message of the President*, 1898, Spain (Washington D.C.: Government Printing Office, 1901), 752.

provide support and comfort, particularly when the disease might be fatal.[47] By disallowing any visits from family members at the cholera hospitals and detention camps, American officials created a situation where Filipinos would often choose to conceal their infections and risk death rather than isolate themselves from their family members. The ineffective treatments offered at the cholera hospitals created similar issues. In hopes of destroying cholera germs, which infect the small intestine, patients at the cholera hospitals were given strong acidic chemicals such as benzozone and diluted sulfuric acid; these treatments were completely ineffective against cholera and destroyed the stomach linings and many patients.[48] The combination of ineffective, forced treatments and high fatality rates in the cholera hospitals further encouraged concealment of cholera and even led to rumors that the Americans were poisoning the sick with strychnine.[49] A final example of American disdain for Filipino culture during the cholera epidemic was the policy of mandatory cremation. As part of General Order No. 66, Major General Chaffee commanded that all cholera dead be buried in hermetically sealed metal coffins or cremated.[50] Filipino historian Reynaldo Ileto writes that "no Filipino in his right mind approved of cremation,"[51] and one American teacher warned that "the natives had such a horror of [cremation] that they would rather scoop out a trench under a house in which to bury the dead, or throw the body in the nearest swamp."[52] Such warnings were not unfounded, as demonstrated by a *Manila Times* article on the bodies of cholera victims washing up in Manila's Pasig River.[53]

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47. Heiser, *Doctor's Odyssey*, 109.

48. Rodney Sullivan, "Cholera and Colonialism in the Philippines, 1899-1903," in *Disease, Medicine, and Empire: Perspectives on Western Medicine and the Experience of European Expansion*, ed. Milton Lewis and Roy Macleod (London: Routledge, 1988), 296.

49. Worcester, *Asiatic Cholera*, 18.

50. "Chaffee's Cholera Order," *Manila Times*, March 27, 1902.

51. Ileto, "American Sanitary Order," 137.

52. "Teachers and Cholera in the Philippines," *The Nation*, October 15, 1903, 298.

53. "Burial in the Pasig River," *Manila Times*, April 5, 1902.

While American anti-cholera policies such as isolation in detention camps and mandatory cremation may have been well-intentioned, their complete incompatibility with Filipino culture often rendered them ineffective and worsened the epidemic by encouraging Filipinos to conceal their sickness.

Anti-cholera Policies, Colonial Power, and the Transition from War to Peace

In addition to reinforcing Filipino racial inferiority, anti-cholera policies gave American officials a way to expand their control over Filipinos in a non-military manner. The 1902-1904 epidemic occurred during a transitionary period of American rule. Until the official conclusion of the Philippine-American War on July 4, 1902, some areas of the country were under direct military control, while others were granted civil governments. Even after the war's end, American officials grappled with questions about how to retain control over areas still sympathetic to the Filipino nationalist cause or simply reluctant to be ruled by another colonial power. During both the end of the Philippine-American War and the beginning of peace, anti-cholera policies such as inspections, selective quarantines, and threats against property reinforced American power in the Philippines without relying on explicit military force.

One anti-cholera policy that rooted out disease while simultaneously enforcing American power was the extensive inspection of Filipino homes. During the height of the Philippine-American War, particularly during the period of intensified guerilla warfare, American soldiers had regularly invaded Filipino dwellings in search of insurgents or supplies destined for rural guerilla bases. During the cholera epidemic, these soldiers were replaced with sanitary inspectors who sought evidence of cholera rather than signs of insurgency. Shortly after the epidemic began, the Board of Health trained more than 1,500 sanitary inspectors, many of them

American teachers.[54] Throughout the day and night, these inspectors entered Filipino homes, searched for signs of cholera, and sent any infected Filipinos to cholera hospitals and detention camps. In one town in Laguna Province, sanitary workers inspected 18,000 homes and then returned to each home a month later. These inspections gave American officials constant opportunities to observe their colonial subjects and regulate their behaviors. While rigorous inspections by sanitary workers may have helped curb cholera, they also gave American officials constant surveillance powers, just as their ability to use military force against Filipinos was withering away.

Strict and selectively enforced quarantines were another anti-cholera policy that reinforced American power over the movement of Filipinos. As soon as Worcester learned of the first cholera cases, he immediately attempted to quarantine Manila and establish a cordon around the Mariquina River, which served as the city's water supply.[55] While any Filipinos attempting to enter or leave the city required special authorization, Americans retained their abilities to move freely, even after cholera cases were discovered among soldiers. Even more surprisingly, American troops continued to sail from Manila to other parts of the archipelago, eventually spreading the epidemic southward via Nueva Caceres. The cordon around the Mariquina River, which roughly 20,000 locals relied upon for water, also proved deadly: one woman was shot after crossing the quarantine line.[56] After the epidemic, even Worcester admitted that the quarantine around Manila was ineffective in stopping cholera, calling it his "most serious blunder" and "entirely futile."[57] Nevertheless, quarantines and cordons provided

54. Bureau of Insular Affairs, *Third Annual Report of the Philippine Commission: 1902*, pt. 2 (Washington D.C.: Government Printing Office, 1903), 919.
55. Worcester, *Asiatic Cholera*, 17.
56. Ong, "Public Health and the Clash of Cultures," 210.
57. Worcester, *Asiatic Cholera*, 24.

another way for American officials to control Filipino movement, even after the end of wartime conditions.

The power of health officials to destroy Filipino property provides an additional example of how the cholera epidemic effectively gave wartime powers to American colonial officials. As mentioned above, Worcester's orders to destroy the entire Farola district demonstrated the complete power of Americans over Filipino property during both war and peace. The threat of a destroyed home also gave American officials an effective means of leverage over Filipinos still hesitant to accept colonial rule. The failure to report even a single cholera case to American officials could have devastating consequences for an entire town. In Dagupan, for example, American sanitary inspectors set fire to several homes after learning their inhabitants had failed to report cholera cases. The blaze quickly grew out of control and threatened to burn the entire town.[58] American soldiers and sanitary inspectors also maintained control of the food supply. During the epidemic, food safety measures were zealously enforced since Filipino cooking was considered unclean. In addition to banning the feasts commonly held during religious festivals, American officials sometimes confiscated food from Filipinos they suspected of smuggling after a cook bypassed the Manila quarantine and spread cholera through smuggled food.[59] While American violations of property were less flagrant during the 1902-1904 epidemic than during the Philippine-American War, Americans continued to exercise control over Filipino homes and food due to anti-cholera measures.

Conclusion

58. *Manila Times*, July 9, 1902.
59. De Bevoise, *Agents of Apocalypse*, 178-79.

While the sanitary policies that American officials created during the 1902-1904 cholera epidemic in the colonial Philippines were ostensibly enforced to counter the spread of disease, they were ultimately more significant for the ways they reinforced American racial understandings of Filipinos as unclean, fatalistic, and immature racial inferiors and extended American control over colonial subjects in a country transitioning from war to peace. Relying on intersecting understandings of the germ theory of disease and racial science, American colonial officials portrayed Filipinos as belonging to a race that was unclean and constantly harboring microbial threats to Americans. Harsh American anti-cholera measures exonerated Americans for their role in the epidemic while simultaneously blaming Filipinos for the spread of disease. Often viewing themselves as benevolent saviors, American officials and medical workers forced Filipinos into crowded detention camps, burned entire neighborhoods, and forcibly administered ineffective treatments to infected Filipinos in cholera hospitals. Additional measures such as selective quarantines, sanitary inspections, and threats against Filipino property ensured the continuing dominance of colonial officials, even after the official end of the Philippine-American War. American anti-cholera policies during the 1902-1904 epidemic may have been well-intentioned, but they undoubtedly reinforced colonial power over Filipinos using theoretical ideas about race and practical methods of control. While this finding may not be entirely surprising, it furthers our understanding of how colonial power often relied on intersecting racial, medical, and military methods. Additionally, it invites further investigations into race in the American-controlled Philippines, such as the role that Spaniards, Chinese, Japanese, and other groups played in the 1902-1904 epidemic.

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